Taipei Veterans General Hospital

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Sample Information

Patient Name: 鄭淑貞
Gender: Female
ID No.: A202321050
History No.: 27424460
Age: 75

Ordering Doctor: DOC466E_司徒皓平
Ordering REQ.: 0CBZVSB
Signing in Date: 2022/11/10

Path No.: M111-00003
MP No.: F22123
Assay: Oncomine Focus Assay
Sample Type: FFPE
Block No.: S111-44971A+B
Percentage of tumor cells: 25%

Reporting Doctor: DOC5466K 葉奕成 (Phone: 8#5466)

Note:

Sample Cancer Type: Non-Small Cell Lung Cancer

Table of Contents

	Page
Variant Details	2
Biomarker Descriptions	2
Relevant Therapy Summary	3
Relevant Therapy Details	4
Clinical Trials Summary	10
Alert Details	10

Report Highlights

1 Relevant Biomarkers
6 Therapies Available
6 Clinical Trials

Relevant Non-Small Cell Lung Cancer Variants

Gene	Finding	Gene	Finding
ALK	None detected	NTRK1	None detected
BRAF	None detected	NTRK2	None detected
EGFR	***EGFR p.(G719A) c.2156G>C***	NTRK3	None detected
ERBB2	None detected	RET	None detected
KRAS	None detected	ROS1	None detected
MET	None detected		

Disclaimer: The data presented here is from a curated knowledgebase of publicly available information, but may not be exhaustive. The data version is 2022.10(005).The content of this report has not been evaluated or approved by FDA, EMA or other regulatory agencies.

Relevant Biomarkers

Tier	Genomic Alteration	Relevant Therapies (In this cancer type)	Relevant Therapies (In other cancer type)	Clinical Trials
IA	*EGFR p.(G719A) c.2156G>C* epidermal growth factor receptor Allele Frequency: 18.15%	**afatinib** [1, 2] **gefitinib*** [2] dacomitinib erlotinib osimertinib	None	6

Public data sources included in relevant therapies: FDA[1], NCCN, **EMA**[2], ESMO
Tier Reference: Li et al. *Standards and Guidelines for the Interpretation and Reporting of Sequence Variants in Cancer: A Joint Consensus Recommendation of the Association for Molecular Pathology, American Society of Clinical Oncology, and College of American Pathologists.* J Mol Diagn. 2017 Jan;19(1):4-23.
*** Includes biosimilars**

Variant Details

DNA Sequence Variants

Gene	Amino Acid Change	Coding	Variant ID	Locus	Allele Frequency	Transcript	Variant Effect	Coverage
EGFR	p.(G719A)	c.2156G>C	COSM6239	chr7:55241708	18.15%	NM_005228.5	missense	2000
JAK1	p.(P733=)	c.2199A>G	.	chr1:65310489	53.83%	NM_002227.4	synonymous	1997
FGFR4	p.(P136L)	c.407C>T	.	chr5:176517797	99.65%	NM_213647.3	missense	2000
EGFR	p.(Q787=)	c.2361G>A	.	chr7:55249063	46.80%	NM_005228.5	synonymous	2000
RET	p.(L769=)	c.2307G>T	.	chr10:43613843	99.95%	NM_020975.6	synonymous	2000

Biomarker Descriptions

EGFR (epidermal growth factor receptor)

Background: The EGFR gene encodes the epidermal growth factor receptor (EGFR) tyrosine kinase, a member of the ERBB/human epidermal growth factor receptor (HER) family. In addition to EGFR/ERBB1/HER1, other members of the ERBB/HER family include ERBB2/HER2, ERBB3/HER3, and ERBB4/HER4[1]. EGFR ligand induced dimerization results in kinase activation and leads to stimulation of oncogenic signaling pathways including the PI3K/AKT/MTOR and RAS/RAF/MEK/ERK pathways. Activation of these pathways promote cell proliferation, differentiation, and survival[2,3].

Alterations and prevalence: Recurrent somatic mutations in the tyrosine kinase domain (TKD) of EGFR are observed in approximately 10-20% of lung adenocarcinoma, and at higher frequencies in never-smoker, female, and Asian populations[4,5,6,7]. The most common mutations occur near the ATP-binding pocket of the TKD and include short in-frame deletions in exon 19 (EGFR exon 19 deletion) and the L858R amino acid substitution in exon 21[8]. These mutations constitutively activate EGFR resulting in downstream signaling, and represent 80% of the EGFR mutations observed in lung cancer. A second group of less prevalent activating mutations include E709K, G719X, S768I, L861Q, and short in-frame insertion mutations in exon 20[9,10,11,12]. EGFR activating mutations in lung cancer tend to be mutually exclusive to KRAS activating mutations[13]. In contrast, a different set of recurrent activating EGFR mutations in the extracellular domain include R108K, A289V and G598V and are primarily observed in glioblastoma[8,14]. Amplification of EGFR is observed in several cancer types including 30% of glioblastoma, 12% of esophageal cancer, 10% of head and neck cancer, 5% of bladder cancer, and 5% of lung squamous cell carcinoma[5,6,7,14,15]. Deletion of exons 2-7, encoding the extracellular domain of EGFR (EGFRvIII), results in overexpression of a ligand-independent constitutively active protein and is observed in approximately 30% of glioblastoma[16,17,18].

Potential relevance: Approved first-generation EGFR tyrosine kinase inhibitors (TKIs) include erlotinib[19] (2004) and gefitinib[20] (2015), which block the activation of downstream signaling by reversible interaction with the ATP-binding site. Although initially approved for advanced lung cancer, the discovery that drug sensitivity was associated with exon 19 and exon 21 activating mutations allowed first-generation TKIs to become subsequently approved for front-line therapy in lung cancer tumors containing exon 19 or exon 21 activating mutations. Second-generation TKIs afatinib[21] (2013) and dacomitinib[22] (2018) bind EGFR and other ERBB/HER gene family members irreversibly and were subsequently approved. First- and second-generation TKIs afatinib, dacomitinib, erlotinib, and gefitinib are recommended for the treatment NSCLC harboring EGFR exon 19 insertions, exon 19 deletions, point mutations L861Q,

Biomarker Descriptions (continued)

L858R, S768I, and codon 719 mutations, whereas most EGFR exon 20 insertions, except p.A763_Y764insFQEA, confer resistance to the same therapies[23,24,25,26]. However, in 2021, the irreversible tyrosine kinase inhibitor, mobocertinib[27]was FDA approved for the treatment of NSCLC with EGFR exon 20 insertion mutations. Additionally, in 2022, the FDA granted breakthrough therapy designation to the irreversible EGFR inhibitors, CLN-081 (TPC-064)[28] and sunvozertinib[29], for locally advanced or metastatic non-small cell lung cancer harboring EGFR exon 20 insertion mutations. In lung cancer containing EGFR exon 19 or 21 activating mutations, treatment with TKIs is eventually associated with the emergence of drug resistance[30]. The primary resistance mutation that emerges following treatment with first-generation TKI is T790M, accounting for 50-60% of resistant cases[8]. Third generation TKIs were developed to maintain sensitivity in the presence of T790M. Osimertinib[31] (2015) is an irreversible inhibitor indicated for metastatic EGFR T790M positive lung cancer and for the first-line treatment of metastatic NSCLC containing EGFR exon 19 deletions or exon 21 L858R mutations. Like first-generation TKIs, treatment with osimertinib is associated with acquired resistance. In this case, resistance is associated with the C797S mutation and occurs in 22-44% of cases[30]. The T790M and C797S mutations may be each selected following sequential treatment with a first-generation TKI followed by a third-generation TKI or vice versa[32]. T790M and C797S can occur in either cis or trans allelic orientation[32]. If C797S is observed following progression after treatment with a third-generation TKI in the first-line setting, sensitivity may be retained to first-generation TKIs[32]. If C797S co-occurs in trans with T790M following sequential treatment with first- and third-generation TKIs, patients may exhibit sensitivity to combination first- and third-generation TKIs, but resistance to third-generation TKIs alone[32,33]. However, C797S occurring in cis conformation with T790M, confers resistance to first- and third-generation TKIs[32]. Fourth-generation TKIs are in development to overcome acquired C797S and T790M resistance mutations after osimertinib treatment. EGFR targeting antibodies including cetuximab (2004), panitumumab (2006), and necitumumab (2016) are under investigation in combination with EGFR-targeting TKIs for efficacy against EGFR mutations. The bispecific antibody, amivantamab[34], targeting EGFR and MET was approved (2021) NSCLC tumors harboring EGFR exon 20 insertion mutations. The Oncoprex immunogene therapy quaratusugene ozeplasmid[35] in combination with osimertinib received a fast track designation from the FDA (2020) for NSCLC tumors harboring EGFR mutations that progressed on osimertinib alone. BDTX-189[36] was granted a fast track designation (2020) for the treatment of solid tumors harboring an EGFR exon 20 insertion mutation.

Relevant Therapy Summary

● In this cancer type ○ In other cancer type ◑ In this cancer type and other cancer types ✖ No evidence

EGFR p.(G719A) c.2156G>C

Relevant Therapy	FDA	NCCN	EMA	ESMO	Clinical Trials*
afatinib	●	●	●	●	✖
gefitinib	✖	●	●	●	✖
dacomitinib	✖	●	✖	●	✖
erlotinib	✖	●	✖	●	✖
osimertinib	✖	●	✖	●	✖
gefitinib (Mylan)	✖	✖	●	✖	✖
atezolizumab, bevacizumab, chemotherapy	✖	✖	✖	✖	● (II)
BLU-945	✖	✖	✖	✖	● (I/II)
sunvozertinib	✖	✖	✖	✖	● (I/II)
amivantamab	✖	✖	✖	✖	● (I)
lazertinib, amivantamab, chemotherapy	✖	✖	✖	✖	● (I)
TNO-155, nazartinib	✖	✖	✖	✖	● (I)

* Most advanced phase (IV, III, II/III, II, I/II, I) is shown and multiple clinical trials may be available.

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Relevant Therapy Details

Current FDA Information

● In this cancer type ○ In other cancer type ◑ In this cancer type and other cancer types

FDA information is current as of 2022-09-14. For the most up-to-date information, search www.fda.gov.

EGFR p.(G719A) c.2156G>C

afatinib

Cancer type: Non-Small Cell Lung Cancer **Label as of:** 2022-04-07 **Variant class:** EGFR G719 mutation

Indications and usage:

GILOTRIF® is a kinase inhibitor indicated for:

- First-line treatment of patients with metastatic non-small cell lung cancer (NSCLC) whose tumors have non-resistant epidermal growth factor receptor (EGFR) mutations as detected by an FDA-approved test.

Limitations of Use: Safety and efficacy of GILOTRIF® were not established in patients whose tumors have resistant EGFR mutations

- Treatment of patients with metastatic, squamous NSCLC progressing after platinum-based chemotherapy

Reference:

https://www.accessdata.fda.gov/drugsatfda_docs/label/2022/201292s017lbl.pdf

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Current NCCN Information

● In this cancer type ○ In other cancer type ◑ In this cancer type and other cancer types

NCCN information is current as of 2022-09-01. For the most up-to-date information, search www.nccn.org.
For NCCN International Adaptations & Translations, search www.nccn.org/global/international_adaptations.aspx.

EGFR p.(G719A) c.2156G>C

● afatinib

Cancer type: Non-Small Cell Lung Cancer **Variant class:** EGFR G719 mutation

NCCN Recommendation category: 2A

Population segment (Line of therapy):
- Adenocarcinoma, Large Cell, Squamous Cell, Not otherwise specified (NOS); Advanced, Metastatic (First-line therapy); Preferred intervention
- Adenocarcinoma, Large Cell, Squamous Cell, Not otherwise specified (NOS); Advanced, Metastatic (Subsequent therapy); Preferred intervention

Reference: NCCN Guidelines® - NCCN-Non-Small Cell Lung Cancer [Version 3.2022]

● dacomitinib

Cancer type: Non-Small Cell Lung Cancer **Variant class:** EGFR G719 mutation

NCCN Recommendation category: 2A

Population segment (Line of therapy):
- Adenocarcinoma, Large Cell, Squamous Cell, Not otherwise specified (NOS); Advanced, Metastatic (First-line therapy); Other recommended intervention
- Adenocarcinoma, Large Cell, Squamous Cell, Not otherwise specified (NOS); Advanced, Metastatic (Subsequent therapy)

Reference: NCCN Guidelines® - NCCN-Non-Small Cell Lung Cancer [Version 3.2022]

● erlotinib

Cancer type: Non-Small Cell Lung Cancer **Variant class:** EGFR G719 mutation

NCCN Recommendation category: 2A

Population segment (Line of therapy):
- Adenocarcinoma, Large Cell, Squamous Cell, Not otherwise specified (NOS); Advanced, Metastatic (First-line therapy); Other recommended intervention
- Adenocarcinoma, Large Cell, Squamous Cell, Not otherwise specified (NOS); Advanced, Metastatic (Subsequent therapy)

Reference: NCCN Guidelines® - NCCN-Non-Small Cell Lung Cancer [Version 3.2022]

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EGFR p.(G719A) c.2156G>C (continued)

gefitinib

Cancer type: Non-Small Cell Lung Cancer **Variant class:** EGFR G719 mutation

NCCN Recommendation category: 2A

Population segment (Line of therapy):
- Adenocarcinoma, Large Cell, Squamous Cell, Not otherwise specified (NOS); Advanced, Metastatic (First-line therapy); Other recommended intervention
- Adenocarcinoma, Large Cell, Squamous Cell, Not otherwise specified (NOS); Advanced, Metastatic (Subsequent therapy)

Reference: NCCN Guidelines® - NCCN-Non-Small Cell Lung Cancer [Version 3.2022]

osimertinib

Cancer type: Non-Small Cell Lung Cancer **Variant class:** EGFR G719 mutation

NCCN Recommendation category: 2A

Population segment (Line of therapy):
- Adenocarcinoma, Large Cell, Squamous Cell, Not otherwise specified (NOS); Advanced, Metastatic (First-line therapy); Preferred intervention
- Adenocarcinoma, Large Cell, Squamous Cell, Not otherwise specified (NOS); Advanced, Metastatic (Subsequent therapy); Preferred intervention

Reference: NCCN Guidelines® - NCCN-Non-Small Cell Lung Cancer [Version 3.2022]

afatinib

Cancer type: Non-Small Cell Lung Cancer **Variant class:** EGFRi sensitizing mutation

NCCN Recommendation category: 2B

Population segment (Line of therapy):
- Brain Metastases (Line of therapy not specified)

Reference: NCCN Guidelines® - NCCN-Central Nervous System Cancers [Version 1.2022]

gefitinib

Cancer type: Non-Small Cell Lung Cancer **Variant class:** EGFRi sensitizing mutation

NCCN Recommendation category: 2B

Population segment (Line of therapy):
- Brain Metastases (Line of therapy not specified)

Reference: NCCN Guidelines® - NCCN-Central Nervous System Cancers [Version 1.2022]

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Current EMA Information

● In this cancer type ○ In other cancer type ◑ In this cancer type and other cancer types

EMA information is current as of 2022-09-14. For the most up-to-date information, search www.ema.europa.eu/ema.

EGFR p.(G719A) c.2156G>C

afatinib

Cancer type: Non-Small Cell Lung Cancer **Label as of:** 2021-04-21 **Variant class:** EGFR G719 mutation

Reference:
https://www.ema.europa.eu/en/documents/product-information/giotrif-epar-product-information_en.pdf

gefitinib

Cancer type: Non-Small Cell Lung Cancer **Label as of:** 2022-07-05 **Variant class:** EGFR G719 mutation

Reference:
https://www.ema.europa.eu/en/documents/product-information/iressa-epar-product-information_en.pdf

gefitinib (Mylan)

Cancer type: Non-Small Cell Lung Cancer **Label as of:** 2022-06-16 **Variant class:** EGFR G719 mutation

Reference:
https://www.ema.europa.eu/en/documents/product-information/gefitinib-mylan-epar-product-information_en.pdf

Disclaimer: The data presented here is from a curated knowledgebase of publicly available information, but may not be exhaustive. The data version is 2022.10(005).

Current ESMO Information

● In this cancer type ○ In other cancer type ◑ In this cancer type and other cancer types

ESMO information is current as of 2022-09-01. For the most up-to-date information, search www.esmo.org.

EGFR p.(G719A) c.2156G>C

● afatinib

Cancer type: Non-Small Cell Lung Cancer **Variant class:** EGFRi sensitizing mutation

ESMO Level of Evidence/Grade of Recommendation: I / A

Population segment (Line of therapy):
- Advanced (First-line therapy)

Reference: ESMO Clinical Practice Guidelines - ESMO-Metastatic Non-Small-Cell Lung Cancer [Online Guideline (15SEP2020 - https://www.esmo.org/guidelines/lung-and-chest-tumours/clinical-practice-living-guidelines-metastatic-non-small-cell-lung-cancer); Ann Oncol (2018) 29 (suppl 4): iv192–iv237.]

● erlotinib

Cancer type: Non-Small Cell Lung Cancer **Variant class:** EGFRi sensitizing mutation

ESMO Level of Evidence/Grade of Recommendation: I / A

Population segment (Line of therapy):
- Advanced (First-line therapy)

Reference: ESMO Clinical Practice Guidelines - ESMO-Metastatic Non-Small-Cell Lung Cancer [Online Guideline (15SEP2020 - https://www.esmo.org/guidelines/lung-and-chest-tumours/clinical-practice-living-guidelines-metastatic-non-small-cell-lung-cancer); Ann Oncol (2018) 29 (suppl 4): iv192–iv237.]

● gefitinib

Cancer type: Non-Small Cell Lung Cancer **Variant class:** EGFRi sensitizing mutation

ESMO Level of Evidence/Grade of Recommendation: I / A

Population segment (Line of therapy):
- Advanced (First-line therapy)

Reference: ESMO Clinical Practice Guidelines - ESMO-Metastatic Non-Small-Cell Lung Cancer [Online Guideline (15SEP2020 - https://www.esmo.org/guidelines/lung-and-chest-tumours/clinical-practice-living-guidelines-metastatic-non-small-cell-lung-cancer); Ann Oncol (2018) 29 (suppl 4): iv192–iv237.]

● osimertinib

Cancer type: Non-Small Cell Lung Cancer **Variant class:** EGFRi sensitizing mutation

ESMO Level of Evidence/Grade of Recommendation: I / A

Population segment (Line of therapy):
- Advanced (First-line therapy); ESMO-MCBS v1.1 score: 4

Reference: ESMO Clinical Practice Guidelines - ESMO-Metastatic Non-Small-Cell Lung Cancer [Online Guideline (15SEP2020 - https://www.esmo.org/guidelines/lung-and-chest-tumours/clinical-practice-living-guidelines-metastatic-non-small-cell-lung-cancer); Ann Oncol (2018) 29 (suppl 4): iv192–iv237.]

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EGFR p.(G719A) c.2156G>C (continued)

dacomitinib

Cancer type: Non-Small Cell Lung Cancer **Variant class:** EGFRi sensitizing mutation

ESMO Level of Evidence/Grade of Recommendation: I / B

Population segment (Line of therapy):
- Advanced (First-line therapy); ESMO-MCBS v1.1 score: 3

Reference: ESMO Clinical Practice Guidelines - ESMO-Metastatic Non-Small-Cell Lung Cancer [Online Guideline (15SEP2020 - https://www.esmo.org/guidelines/lung-and-chest-tumours/clinical-practice-living-guidelines-metastatic-non-small-cell-lung-cancer); Ann Oncol (2018) 29 (suppl 4): iv192–iv237.]

erlotinib

Cancer type: Non-Small Cell Lung Cancer **Variant class:** EGFRi sensitizing mutation

ESMO Level of Evidence/Grade of Recommendation: III / B

Population segment (Line of therapy):
- Non-squamous Cell (Maintenance therapy)

Reference: ESMO Clinical Practice Guidelines - ESMO-Metastatic Non-Small-Cell Lung Cancer [Online Guideline (15SEP2020 - https://www.esmo.org/guidelines/lung-and-chest-tumours/clinical-practice-living-guidelines-metastatic-non-small-cell-lung-cancer); Ann Oncol (2018) 29 (suppl 4): iv192–iv237.]

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Clinical Trials in Taiwan region:

Clinical Trials Summary

EGFR p.(G719A) c.2156G>C

NCT ID	Title	Phase
NCT04147351	A Phase II Study of Atezolizumab in Combination With Bevacizumab, Carboplatin or Cisplatin, and Pemetrexed for EGFR-mutant Metastatic Non-small Cell Lung Cancer Patients After Failure of EGFR Tyrosine Kinase Inhibitors.	II
NCT02609776	A Phase I, First-in-Human, Open-Label, Dose Escalation Study of JNJ-61186372, a Human Bispecific EGFR and cMet Antibody, in Subjects With Advanced Non-Small Cell Lung Cancer.	I
NCT04862780	A Phase I/II Study Targeting Acquired Resistance Mechanisms in Patients With EGFR Mutant Non-Small Cell Lung Cancer.	I/II
NCT03114319	An Open-label, Multi-center, Phase I, Dose Finding Study of Oral TNO155 in Adult Patients With Advanced Solid Tumors.	I
NCT03974022	A Phase I/II, Open-Label, Multicenter Study to Assess the Safety, Tolerability, Pharmacokinetics and Anti-tumor Efficacy of DZD9008 in Patients With Advanced Non-Small Cell Lung Cancer (NSCLC) with EGFR or HER2 Mutation	I/II
NCT04077463	An Open-label Phase I/Ib Study to Evaluate the Safety and Pharmacokinetics of JNJ-73841937 (Lazertinib), a Third Generation EGFR-TKI, as Monotherapy or in Combinations With JNJ-61186372, a Human Bispecific EGFR and cMet Antibody in Participants With Advanced Non-Small Cell Lung Cancer	I

Alerts Informed By Public Data Sources

Current FDA Information

Contraindicated Not recommended Resistance Breakthrough Fast Track

FDA information is current as of 2022-09-14. For the most up-to-date information, search www.fda.gov.

EGFR p.(G719A) c.2156G>C

patritumab deruxtecan

Cancer type: Non-Small Cell Lung Cancer **Variant class:** EGFR G719 mutation

Supporting Statement:
The FDA has granted Breakthrough Therapy Designation to a potential first-in-class HER3 directed antibody-drug conjugate, patritumab deruxtecan, for metastatic or locally advanced, EGFR-mutant non-small cell lung cancer.

Reference:

https://www.cancernetwork.com/view/fda-grants-breakthrough-therapy-status-to-patritumab-deruxtecan-for-egfr-metastatic-nsclc

Disclaimer: The data presented here is from a curated knowledgebase of publicly available information, but may not be exhaustive. The data version is 2022.10(005).

EGFR p.(G719A) c.2156G>C (continued)

osimertinib + quaratusugene ozeplasmid

Cancer type: Non-Small Cell Lung Cancer **Variant class:** EGFR mutation

Supporting Statement:
The FDA has granted Fast Track Designation to the immunogene therapy, quaratusugene ozeplasmid, in combination with EGFR inhibitor osimertinib for the treatment of non-small cell lung cancer (NSCLC) with EFGR mutations that progressed after treatment with osimertinib alone.

Reference:

https://www.genprex.com/news/genprex-receives-u-s-fda-fast-track-designation-for-gene-therapy-that-targets-lung-cancer/

Current NCCN Information

Contraindicated Not recommended Resistance Breakthrough Fast Track

NCCN information is current as of 2022-09-01. For the most up-to-date information, search www.nccn.org.
For NCCN International Adaptations & Translations, search www.nccn.org/global/international_adaptations.aspx.

EGFR p.(G719A) c.2156G>C

atezolizumab

Cancer type: Non-Small Cell Lung Cancer **Variant class:** EGFR mutation

Summary:

NCCN Guidelines® include the following supporting statement(s):

- "subsequent therapy with pembrolizumab, nivolumab, or atezolizumab is not recommended in patients with EGFR mutations or ALK rearrangements."

Reference: NCCN Guidelines® - NCCN-Non-Small Cell Lung Cancer [Version 3.2022]

nivolumab

Cancer type: Non-Small Cell Lung Cancer **Variant class:** EGFR mutation

Summary:

NCCN Guidelines® include the following supporting statement(s):

- "subsequent therapy with pembrolizumab, nivolumab, or atezolizumab is not recommended in patients with EGFR mutations or ALK rearrangements."

Reference: NCCN Guidelines® - NCCN-Non-Small Cell Lung Cancer [Version 3.2022]

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EGFR p.(G719A) c.2156G>C (continued)

pembrolizumab

Cancer type: Non-Small Cell Lung Cancer **Variant class:** EGFR mutation

Summary:

NCCN Guidelines® include the following supporting statement(s):

- "subsequent therapy with pembrolizumab, nivolumab, or atezolizumab is not recommended in patients with EGFR mutations or ALK rearrangements."

Reference: NCCN Guidelines® - NCCN-Non-Small Cell Lung Cancer [Version 3.2022]

Disclaimer: The data presented here is from a curated knowledgebase of publicly available information, but may not be exhaustive. The data version is 2022.10(005).

Signatures

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Laboratory Supervisor: 2022.11.11 13:59:01 +08'00'

Pathologist: Yi-Chu Yeh 2022.11.11 13:59:15 +08'00'

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